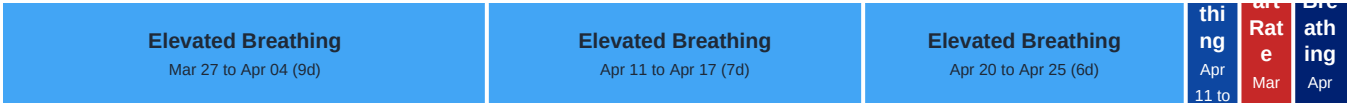


Patient ID: EGPatient ID: EGPeriod: March 26 to April 25, 2024Report Date: April 26, 2024Coverage: Bed: 552/744h (74.2%)Chair: 395/744h (53.1%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Mar 26 to Apr 25

■ HR ■ Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	60 bpm	49 bpm	74 bpm
Breathing	18 brpm	11 brpm	46 brpm

Last 24 hours: one high breathing episode in the afternoon; one elevated breathing episode in the evening.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 25	5 PM to 6 PM	2h	2	Elevated Breathing	29	29	30	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 25	4 PM to 5 PM	1h	1	High Breathing (brief)	38	31	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

29 episodic events Detected, spanning 35h total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate, Very High Breathing.
Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
29	35h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 16	11 AM to 12 PM	1h	1	Very High Breathing	46	46	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Apr 11	10 AM to 11 AM	2h	2	High Breathing	38	22	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 29	4 AM to 6 AM	3h	2	Low Heart Rate	44	41	46	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 27	12 PM to 2 PM	11h	8	Elevated Breathing	27	19	40	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 11	2 PM to 3 PM	11h	9	Elevated Breathing	26	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 20	6 PM to 7 PM	7h	7	Elevated Breathing	28	16	44	Check SpO2 and lung sounds; assess for fluid overload or early infection

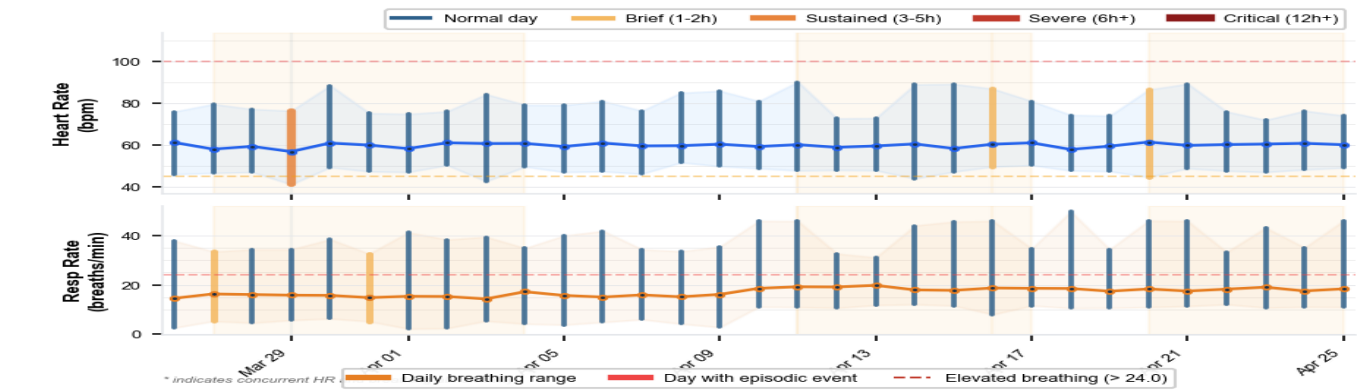
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 40 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- High Breathing (avg 38 brpm, peak 46 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 46 brpm, peak 46 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 16 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

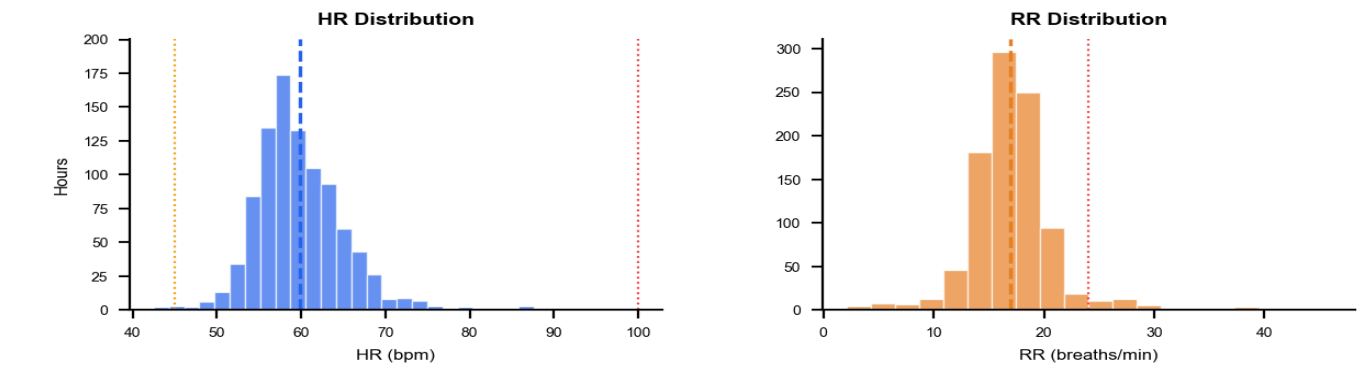
Clinical Pattern Observations:

- 1. **Breathing variability:** RR spread 10 brpm (11 to 22).
- 2. **Clustered pattern:** Episodic events on 5 of 31 days (16%).
- 3. **Daytime pattern:** Episodes concentrated during waking hours.

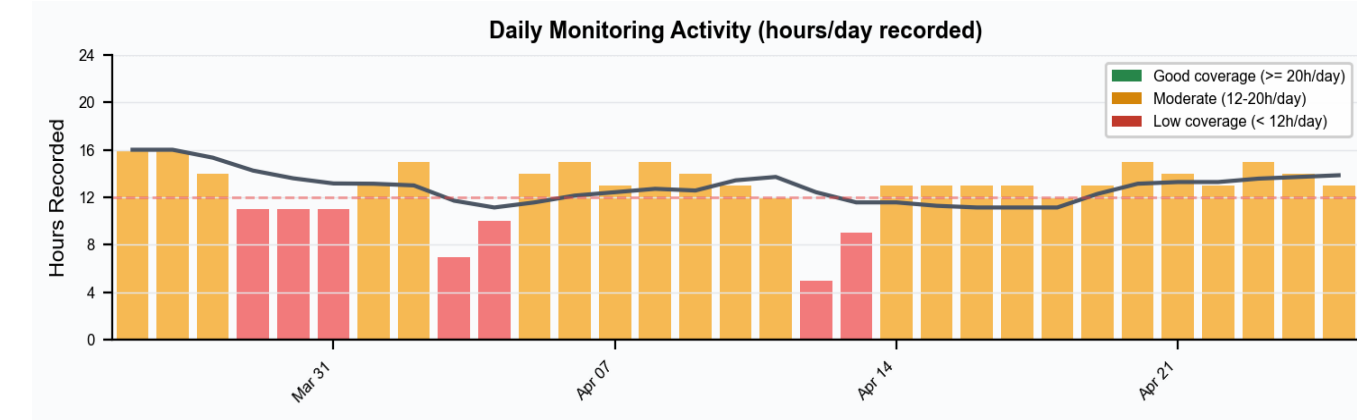


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

■ Very Low HR < 40 bpm ■ Low HR < 45 bpm ■ Elevated HR > 95 bpm ■ High HR > 100 bpm
■ Very High HR > 110 bpm ■ Elevated Breathing > 24 brpm ■ High Breathing > 30 brpm ■ Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.